

Patient 5**Report A**

6503478. 20/05/2016 MR Head

6503478 20/05/2016 MR MRA

Clinical history:

Admitted with fall, R facial droop and dysarthria. Found to have internal capsule/thalamus bleed. MRI to assess further.

Findings:

Comparison is made with the CT study 20/05/2016.

The parenchymal haematoma centred on the left thalamus is again shown, with a small amount of surrounding oedema. Mild local mass effect is evident, with minimal midline shift to the right. Allowing for differences in modality, there has been no significant change in ventricular dimensions.

There is no significant burden of small vessel disease, and no additional microhaemorrhages are demonstrated. The TOF MRA demonstrates normal appearances of the intracranial arteries. Specifically, no vascular malformation is shown underlying the haematoma.

Report B

For Thrombolysis? Yes

Time 1st called: 217am

Time plain CT available on PACS: Unknown

Time of verbal report to stroke registrar: 230

Indication: Right facial weakness and expressive dysphasia at 00.30

OT HEAD Haemorrhage: Yes, there is a small acute intraparenchymal haematoma centred on the lateral aspect of the left thalamus and adjacent posterior limb of the left internal capsule. There is mild amount of surrounding oedema, but no significant midline shift. intravascular thrombus: No

Acute infarct: No Other findings and comments: The remainder of intracranial appearances are unremarkable. In particular, there is no significant burden of Small vessel disease.

CT ANGIOGRAM

Window settings: (intraluminal HU=230; W=460; L=180) (ICA Stenoses NASCET method, measurements on axial 1mm sections) Aortic arch and subclavian arteries: Minor soft plaque is noted at the origin of the left subclavian artery.

R CCA/ICA Normal

L. CCA/ICA: Normal

RVA: Normal

L VA: Aside from calcification at the origin, normal. Intracranial Arteries: Normal. In particular, no vascular malformation is shown in relation to the aforementioned haematoma.

CONCLUSION:

Small acute left thalamichaeatoma.

